

1CD-0-3

carcinoma, infiltrating lobular, NOS 8520/3

Site: breast, NOS C50.9 lw 5/24/11

Criteria	lw 5/24/11	Yes	No
Diagnosis Discrepancy			
Primary Tumor Site Discrepancy			
HIPAA Discrepancy			
Prior Malignancy History			
Dual/Synchronous Primary Noted	Bilateral		
Case is (circle):	QUALIFIED / DISQUALIFIED		
Reviewer Initials	MG	Date Reviewed	5/19/11

***** MODIFIED REPORT - REVIEW ADDENDUM SECTION *****

DIAGNOSIS

(A) SENTINEL LYMPH NODE #1, RIGHT, NOT BLUE:

One lymph node, no tumor present (0/1).

Cytokeratin immunohistochemical stain is negative.

(B) SENTINEL LYMPH NODE #2, RIGHT:

One lymph node, no tumor present (0/1).

Cytokeratin immunohistochemical stain is negative.

(C) SENTINEL LYMPH NODE #3, RIGHT:

One lymph node, no tumor present (0/1).

Cytokeratin immunohistochemical stain is negative.

(D) ~~RIGHT~~ BREAST SEGMENTAL MASTECTOMY:

INVASIVE LOBULAR CARCINOMA OF THE BREAST, MODIFIED BLACK'S NUCLEAR GRADE 1,
WELL DIFFERENTIATED.

INVASIVE LOBULAR CARCINOMA MEASURES 1.7 CM IN MAXIMUM DIMENSION.

INVASIVE LOBULAR CARCINOMA IS 5 MM FROM THE SUPERFICIAL MARGIN OF
RESECTION, 5 MM FROM THE LATERAL MARGIN OF RESECTION, AND 5 MM
FROM THE INFERIOR MARGIN OF RESECTION.

All other margins widely free of tumor.

Atypical lobular hyperplasia.

Fibrocystic changes.

Changes consistent with prior biopsy site.

(E) SECOND SUPERFICIAL INFERIOR LATERAL MARGIN:

Fibrocystic changes.

(F) ADDITIONAL INFERIOR MARGIN:

Atypical lobular hyperplasia.

Fibrocystic changes.

(G) ~~LEFT~~ BREAST EXCISIONAL BIOPSY: — Bilateral

LOBULAR CARCINOMA IN SITU, CLASSIC TYPE.

Changes consistent with prior biopsy site.

Fibrocystic changes.

UUID:100E842E-0ADD-4E58-B318-0C67DC175787
TCGA-GM-A2D0-01A-PR

Redacted

Entire report and diagnosis completed by

COMMENT

Prognostic/predictive markers were performed on the patient's diagnostic biopsy

GROSS DESCRIPTION

(A) SENTINEL LYMPH NODE #1, NOT BLUE, IN VIVO 259, EX VIVO 332 - A tan-pink lymph node is 0.8 x 0.6 x 0.6 cm. The specimen is serially sectioned and two touch preps are performed. Total A.

*FS/DX: ONE LYMPH NODE, NO TUMOR PRESENT ON TOUCH PREPS.

(B) SENTINEL LYMPH NODE #2, RIGHT, BLUE, IN VIVO 21, EX VIVO 17 - A tan-pink lymph node with blue dye is 0.8 x 0.5 x 0.5 cm. The specimen is serially sectioned and two touch preps are performed. Total B.

*FS/DX: ONE LYMPH NODE, NO TUMOR PRESENT ON TOUCH PREPS.

(C) SENTINEL LYMPH NODE #3, RIGHT, NOT BLUE, IN VIVO 50, EX VIVO 71 - A tan-pink lymph node is 0.7 x 0.5 x 0.5 cm. The specimen is serially sectioned and two touch preps are performed. Total C.

*FS/DX: ONE LYMPH NODE, NO TUMOR PRESENT.

(D) RIGHT SEGMENTAL MASTECTOMY, SHORT STITCH SUPERIOR, LONG STITCH MEDIAL - An oriented right segmental mastectomy specimen (6.0 x 6.0 x 3.0 cm). The specimen is oriented as short stitch superior, long stitch medial.

The specimen is serially sectioned sequentially from lateral to medial aspect in seven slices. The cut surface reveals the most lateral aspect (slice #1) towards medial aspect (slice #5), a white firm lesion (1.5 x 1.0 cm). The lesion is distant approximately 0.5 cm from closest to superficial margin, 0.4 cm from closest lateral margin, 0.4 cm from closest inferior margin, 1.5 cm from closest superior margin, and 2.0 cm from closest deep margin.

INK CODE: Superficial - yellow, superior - blue, inferior - green, medial and lateral - red, and deep margin - black.

SECTION CODE: Slice #1, most lateral aspect, D1-D3, in perpendicular sections; slice #2, D4, tumor including superficial margin; D5, superior margin; D6, inferior margin; D7, deep margin; slice #3, D8, tumor including superficial margin; D9, section including superior and deep margin; D10, section including superficial and inferior margin; slice #4; D11, tumor including superficial margin; D12, section including superior and deep margin; D13, section including inferior and deep margin; D14, section including deep margin; slice #5; D15, tumor including superficial and superior margin; D16, section including superior and deep margin; D17, section including superficial and inferior margin, slice #6; D18, section adjacent to the tumor, slice #7; D19, D20, representative sections in perpendicular, most medial aspect.

(E) SECOND SUPERFICIAL INFERIOR AND LATERAL MARGINS CLIPS ON TRUE MARGIN, SHORT STITCH ANTERIOR, LONG STITCH SUPERIOR - An oriented irregular fragment of adipose tissue 4.0 x 4.0 x 0.8 cm. The specimen is oriented as clip on true margin, short stitch anterior, long stitch superior. The specimen is serially sectioned sequentially from superior to inferior.

INK CODE: Anterior-yellow, true margin-black.

SECTION CODE: E1-E7, entirely submitted from superior to inferior.

(F) ADDITIONAL INFERIOR MARGIN CLIPS MARK TRUE MARGIN - An oriented irregular fragment of adipose tissue (3.5 x 2.0 x 0.8 cm). The specimen is oriented as clip on true margin. The true margin is inked in black. The specimen is serially sectioned and entirely submitted in F1-F4.

(G) LEFT EXCISIONAL BIOPSY, SHORT STITCH SUPERIOR, LONG STITCH MEDIAL - An oriented left excisional biopsy specimen (3.0 x 3.0 x 2.0 cm). The specimen is oriented as short stitch superior, long stitch medial.

The specimen is serially sectioned sequentially from medial to lateral aspect in six slices. The cut surface reveals fibrous area and also on slice #3, a surgical coil is present, distant approximately 0.5 cm from closest superior margin, 0.7 cm from closest superficial margin and 0.5 cm from closest deep margin. No tumor is grossly identified.

INK CODE: Superficial - yellow, superior - blue, inferior - green, medial and lateral - red, and deep margin - black.

SECTION CODE: Slice #1, most medial aspect, entirely submitted in perpendicular in G1-G3. Slice #2, G4, superior aspect including margin; G5, inferior aspect including margin; slice #3, G6, superior aspect including margin; G7, inferior aspect including margin; slice #4, G8, superior aspect, including margin; G9, inferior aspect, including margin; slice #5, G10, superior aspect including margin; G11, inferior aspect including margin; slice #6, most lateral aspect; G12, G13, entirely submitted in perpendicular.

CLINICAL HISTORY

Bilateral segmental.

SNOMED CODES

"Some tests reported here may have been developed and performance characteristics determined by
These tests have not been specifically cleared or approved by the U.S. Food and Drug Administration."

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Start of ADDENDUM

ADDENDUM

Addendum completed by

This report is issued to give immunohistochemistry results. Immunohistochemical staining is performed on a representative formalin-fixed, paraffin-embedded section of INVASIVE LOBULAR CARCINOMA, right breast, block D4.

MARKER	RESULTS	% POSITIVE	SCORE
Estrogen Receptor	Positive	90 %	N/A
Progesterone Receptor	Positive	80 %	N/A
HER-2/neu overexpression	Negative	0 %	
Ki-67	High (>35%)	65 %	N/A

FOOTNOTE

Estrogen receptor was assessed by immunohistochemistry using antibody 6F11

Progesterone receptor was assessed by immunohistochemistry using antibody PgR1294

HER-2/neu was assessed by immunohistochemistry using antibody AB8

Ki-67 was assessed by immunohistochemistry using antibody MIB1

Released by:

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